SAMPLE ANSWER · DISTAL RADIUS FRACTURE · CASE 3

Sample 7-minute ISBAR: Joan

Joan Kelly · 84-year-old woman · Fracture outreach home visit, public hospital service · about 1043 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

72 words

"Hi, I'm the occupational therapy student completing today's fracture outreach home visit with Joan Kelly. Joan is 84 and had a left distal radius fracture treated in ED with closed reduction and a below-elbow cast four days ago.

"I need to speak with you urgently, because Joan fell last night and lay on the floor for several hours, she seems confused, and the fingers of her casted hand are swollen and cool."

S Situation

155 words

"I came to review Joan's personal care, meals and cast care. She told me she slipped getting out of bed to go to the toilet 'sometime after one' last night, couldn't push herself up because of her arm, and 'had a little rest on the floor' until it got light. She's walking stiffly and holding her right hip, and there's a damp patch beside the bed.

"The fingers of her left hand are puffy, dusky and cooler than her right, with pins and needles, and the cast is cracked across the back of the wrist. She guessed two different days when I asked what day it was, yesterday's sandwiches are untouched, and she hasn't taken her tablets since she came home because she can't open the bottles.

"Joan says she's fine and won't go back to 'that hospital'. Rhonda, her neighbour, is leaving for work, and I'm due at my next visit in 30 minutes."

B Background

153 words

"Joan is right-handed, widowed and lives alone in a lowset house with her cat, Biscuit. Before this fall, she shopped by bus and cooked for herself. She fell on her outstretched hand at the shops four days ago, and the ED summary says she mobilised independently without a gait aid, was alert and oriented, and declined community services. Her son, Michael, lives in London and is on the phone now.

"The three sealed bottles on her table are labelled for blood pressure and her heart.

"Rhonda says Joan has 'always been a bit forgetful', but we have no documentation of her usual memory and thinking, so I can't tell whether today is different. I also don't know whether she hit her head, exactly how long she was on the floor, her full medication list, whether any tablets thin the blood, whether she has a personal alarm, or whether anyone could stay with her."

A Assessment

307 words

"I'm concerned about several problems that may be linked, and some are time-critical.

"First, the long lie. Lying on the floor from after 1 am until daylight puts an older person at risk of dehydration, hypothermia, pressure injury and muscle breakdown, which can affect the

kidneys. She's holding her hip, so a new injury needs to be excluded, and if she's on a blood thinner, an unwitnessed fall makes a possible head injury more concerning.

"Second, her hand. Swollen, cool, dusky fingers with pins and needles, and a cracked cast after a second fall, may indicate that circulation or nerve function is compromised, the cast is no longer fitting safely, or the fracture position has changed. That needs urgent review.

"Third, her confusion. Not knowing the day, not eating and not recognising the seriousness of her situation may be her usual forgetfulness, but after a long lie, possible dehydration and missed medications, I'd be worried about delirium. The ED documented her as alert and oriented four days ago, so I can't assume this is her baseline.

"Fourth, four days without her blood pressure and heart tablets could matter medically, and the cause is functional: she can't open the bottles one-handed.

"Occupationally, a fracture of her non-dominant hand has exposed how thin her safety margin was. She can't get up from the floor, can't access her medications, isn't eating and is managing night-time toileting alone. The ED's 'independent' was a snapshot that no longer applies.

"Her refusal matters and I want to respect it, but I can't judge whether she can make that decision today, especially if she might be delirious; that needs medical assessment. The grocery order, feeding Biscuit and the crossword are understandable, but they aren't today's priority.

"So my priority is urgent medical review today, and not leaving Joan alone until there's a safe plan."

R Recommendation

356 words

"First, I'd stay with Joan, keep her warm and seated comfortably with her left arm supported on a pillow, and not leave her alone while we arrange review. If she becomes drowsier, more confused or unwell, I'd call triple zero.

"Second, I'd like you to come in now and help me arrange same-day medical assessment, ideally by discussing her with the ED or the outreach medical officer and organising transport. If Joan still declines, I'd ask that the medical team assess her decision-making, rather than us either forcing her or simply leaving.

"I'd also explore why she doesn't want to go back, perhaps fear or worry about Biscuit, and ask Rhonda whether she could feed him.

"For the MDT: medical staff to assess injury from the fall, delirium, the effects of the long lie and the missed medications; orthopaedics to review the cast and circulation in her hand; nursing to monitor observations and pressure areas; pharmacy to supply medications in accessible packaging, such as a dose administration aid; physiotherapy to assess mobility and getting up from the floor; and social work to contact Michael, with Joan's consent, and coordinate supports.

"For OT assessment, once she's medically reviewed, my principal short-term assessment would be observed performance of essential daily tasks within her cast precautions, specifically getting out of bed and to the toilet at night, opening her medication packaging, preparing a simple meal and dressing, because that's where she's struggling. Longer term, I'd use the COPM to capture her own priorities, which might include staying at home with Biscuit and getting back to her shopping.

> "Before any return home, I'd look at night lighting, toilet access or a bedside commode, a personal alarm she'll wear, and a My Aged Care referral for help with meals and personal care. If she needs a period of support after hospital, the Transition Care Program could be considered. As this looks like a fragility fracture followed by another fall, I'd recommend falls assessment and bone-health follow-up through her GP or a fracture liaison service.
>
> "Today, the priority is that Joan is medically reviewed and isn't left alone without a safe plan."

Coaching notes *not spoken*

The immediate priority

A long lie of several hours, possible new confusion, missed cardiac and blood pressure medications, and swollen, cool, dusky fingers in a cracked cast all need same-day medical review. Joan should not be left alone without a plan.

Distractors in this case

- **ED summary: "independent", "alert and oriented", "declined services"** — four days old and predates last night's fall.
- **"She's always been a bit forgetful"** — Rhonda's view doesn't establish baseline or exclude delirium.
- **Grocery order, feeding Biscuit and the crossword** — kind requests, but not the clinical priority (though caring for the cat may help Joan accept review).
- **Joan's refusal** — important to respect and explore, but decision-making today needs medical assessment, not student judgement.

Missing information to clarify

- Usual cognition and function (from Michael, Rhonda or the GP).
- Whether she hit her head; exact time on the floor.
- Full medication list, including any blood thinners.
- Personal alarm, supports and who could stay with her.

Useful references

- Australian Commission on Safety and Quality in Health Care, *Delirium Clinical Care Standard*.
- Australian Commission on Safety and Quality in Health Care, *Preventing Falls and Harm from Falls in Older People: Best Practice Guidelines for Australian Community Care*.

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